ADR IDENTIFICATION BASICS

Task 1 • Preliminary case analysis

Scope note: This analysis uses only the patient details visible in the VirtualWorks Lab task banner and the tutorial topics reviewed. Details not provided in the source material are marked as unknown; none have been invented.

1. Patient and event information

Item	Information available
Patient	Female, 45 years
Suspected medicine	Amoxicillin
Reported symptoms	Rash, itching, nausea and dizziness
Timeline shown	Amoxicillin started → symptoms began → symptoms persisted
Exact dates and duration	Not provided
Dose, route and frequency	Not provided
Indication for amoxicillin	Not provided
Concomitant medicines / allergy history	Not provided
Management and outcome	Not provided

2. Description of the suspected adverse reactions

The task banner lists rash, itching, nausea and dizziness in a 45-year-old female patient in association with amoxicillin use. Rash and itching may be consistent with a cutaneous hypersensitivity-type reaction. Nausea is a recognised gastrointestinal adverse effect of amoxicillin. Dizziness is nonspecific and requires further clinical context. The available information does not establish that all four symptoms have the same mechanism or cause.

3. Medication and event chronology

The only chronology visible in the task banner is: (1) amoxicillin was started; (2) symptoms began; and (3) symptoms persisted. Exact start and onset dates, latency, duration, whether the medicine was continued or stopped, and the course after withdrawal are not supplied.

4. Identification of the suspected medicine

Suspected medicine: Amoxicillin. It is identified as the suspected medicine because it is the medicine named in the scenario and the symptoms are shown as beginning after treatment was started. This temporal sequence supports investigating a possible medicine-related event, but it does not prove causality.

5. Preliminary clinical and pharmacovigilance assessment

Possible reaction pattern: Rash and itching raise the possibility of a hypersensitivity reaction. However, the case does not provide the morphology or distribution of the rash, mucosal involvement, swelling, breathing symptoms, blood pressure, examination findings, or laboratory results. A specific hypersensitivity type or clinical diagnosis therefore cannot be assigned.

Causality: Indeterminate from the information available. The temporal sequence is relevant, but alternative causes, previous exposure, concomitant medicines, dechallenge information and clinical assessment are missing. A formal causality category should not be assigned from the banner alone.

Seriousness: Cannot be assessed from the available information. No outcome or regulatory seriousness criteria are described. Severity (intensity of symptoms) and seriousness (regulatory outcome criteria) are distinct and should not be used interchangeably.

Outcome: Symptoms are described as persisting in the banner; the final outcome is unknown.

6. Information required for follow-up

• Amoxicillin indication, formulation, dose, route, frequency, start date and last dose.
• Exact onset date/time for each symptom and whether symptoms occurred together.
• Detailed description of rash and itching, including distribution, progression and any mucosal or systemic features.
• Relevant medical history, previous exposure to penicillins/other beta-lactams, and known drug allergies.
• All concomitant prescription medicines, OTC medicines, herbal products and supplements.
• Whether amoxicillin was continued, reduced or withdrawn; any treatment given; and response after the action taken.
• Clinical examination, investigations, healthcare professional assessment and final outcome.

7. Conclusion

Amoxicillin is the suspected medicine in this scenario because the task presents rash, itching, nausea and dizziness after its initiation. The symptoms warrant assessment and appropriate documentation. Based on the limited information displayed, causality and seriousness cannot be determined, and a definitive allergic diagnosis should not be made. Additional patient, medication, chronology, clinical and outcome information is required for a complete pharmacovigilance assessment.

8. Key learning applied from the tutorial

This case applies the tutorial's core pharmacovigilance approach: review the patient and medication history, establish the chronology, identify the suspected medicine, distinguish an adverse event from a confirmed adverse drug reaction, consider causality without overstating certainty, and document missing information for follow-up.

Educational limitation: This is a preliminary analysis of the limited scenario displayed in the task banner. It is not a completed ICSR, a regulatory submission, a diagnosis, or medical advice.

References for terminology and further verification

1. World Health Organization. Pharmacovigilance. https://www.who.int/teams/regulation-prequalification/regulation-and-safety/pharmacovigilance
2. WHO–Uppsala Monitoring Centre. Causality assessment. https://who-umc.org/causality-assessment/
3. Indian Pharmacopoeia Commission. Pharmacovigilance Programme of India. https://ipc.gov.in/